

An open angle can still be obstructed

Anterior segment examination distinguishes several secondary causes that share raised pressure.

Compare the obstructing process

Cause	Clinical mechanism
Pseudoexfoliation	Whitish material deposits on lens capsule and trabecular meshwork
Pigment dispersion	Iris pigment release deposits pigment in the trabeculum
Phacolytic glaucoma	Lens proteins leak from a mature cataract and obstruct outflow
Steroid response or uveitis	Trabecular changes or inflammatory material increase resistance

Use the lens and iris clues

Pseudoexfoliation and pigmentary disease are secondary open-angle processes, despite their different deposited material. Phacolytic disease must be separated from phacomorphic lens enlargement causing angle closure. Uveitis may obstruct an open angle, but posterior synechiae can instead produce pupillary block. Gonioscopy, the anterior chamber and the lens therefore help distinguish causes that cannot be separated by tonometry alone.

Rubeosis needs more than pressure drops

Neovascular glaucoma requires treatment of the neovascular drive and assessment of established angle damage.

Match intervention to the problem

Which component needs treatment?

Retinal ischaemic drive

Treat the underlying cause; PRP and intravitreal anti-VEGF address neovascularisation.

Raised IOP

Use appropriate pressure-lowering agents and assess the need for surgical control.

Established synechial closure

Anti-VEGF does not reverse an already closed fibrotic angle.

Inspect, then follow the mechanism

Look for new iris vessels and assess the angle, pressure and retinal disease. An iridotomy directed at pupil block is not a substitute for treating fibrovascular obstruction. Pressure and neovascularisation are linked problems but need distinct treatments. Specialist options for refractory pressure include drainage procedures or ciliary-body-directed treatment, selected according to the eye. Avoid relying on a fixed calendar interval after a retinal event to decide whether rubeosis should be sought.